2026

Analyst Lab Africa

Experience Lab Internship

HealthConnect: Analytics Testing & Refinement Report

Week 7: Testing, Refinement & End-to-End Validation

Data Analytics Track

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Week 7: ANALYTICS TESTING & REFINEMENT REPORT

1. Project Overview

HealthConnect is the Experience Lab project on improving patient appointment attendance and healthcare support using data and AI. This report covers Week 7 for the Data Analytics track: testing the Week 6 work, fixing what failed, and deciding which findings are safe to pass on to the wider solution. The Week 6 work is a five-page Power BI dashboard (Executive, Patient, Clinical, Financial, Operational), an Excel analysis workbook and a PostgreSQL script that answers the 40 assignment questions, all built on the HealthConnect Rwanda database. Week 6 items that were not submitted last week are carried in Appendix A.

Every dashboard card and table was recalculated from the original source files by two routes, a Python notebook (pandas) and a PostgreSQL script, and compared with the dashboard. Amounts are in Rwandan francs (RWF), as set in the Countries reference table. A test marked **Pending** has not been run against the dashboard yet and is not counted as a pass. “Poor” satisfaction means a rating of 1 or 2.

Headline results

Area	Result
Accuracy	29 of 29 dashboard cards, 105 of 105 matrix cells and 4 of 4 chart distributions agree with source at the precision displayed. The 20 facility rows add up exactly to billed RWF 771,133,439, received RWF 694,551,320 and outstanding RWF 76,582,119. Two slicer tests are still Pending (Part C).
Issues	Eight issues (R1 to R8) were found, fixed and retested, including an integrity error in the working source file. Seven refinements (O1 to O7) remain open.
Scope	The dashboard covers 5,000 appointments, which is 8.3% of the 60,000 bills in the billing file. On the two measures compared, average bill (RWF 154,227 against 155,157) and collection rate (90.07% against 90.1%), the 5,000 look representative.
Findings	About 9.9% of billed value is outstanding on a cash basis. Payment status disagrees with the amounts paid, so it cannot measure collection. Half of insurance claims are pending (RWF 204.4M of 407.6M) and none is recorded as rejected. Satisfaction averages 2.93 out of 5, with 40.9% poor, from 35.1% of appointments.
Changed view	Differences between departments, facilities, providers and diagnoses are mostly within normal random variation. The Week 6 working view that staff should follow workload is withdrawn (Part G).

2. Part A: Testing Approach and Scope

Component tested. The Week 6 outputs: dashboard KPIs and visuals, the SQL script, and the findings drawn from them.
Objective. To establish whether dashboard values agree with the underlying data, whether filters and totals behave, and whether the main conclusions hold across segments.

Method.

1. **Independent recalculation.** Each KPI was rebuilt from the original files, not from the Power BI model, in a Python notebook and in PostgreSQL. Both routes gave identical values.
2. **Scope rule.** The 5,000 appointments are the anchor. Billing, payment, claim, feedback, laboratory, prescription, admission and referral rows count only if they link to one of those appointments, the same rule the Power BI model applies. Part F reconciles this to the full files.
3. **Comparison rule.** A value passes when it equals the source at the precision displayed. A card showing 14.3 passes against 14.2857. This tests what the reader sees, not hidden decimals.
4. **Reconciliation.** Facility rows must add up to the card totals, and chart shares must equal source shares.
5. **Segment and significance tests.** Key findings were re-tested across segments with chi-square or Kruskal–Wallis tests at the 5% level (Part E).
6. **Issue handling.** Every mismatch was logged, fixed and retested (Part D).

Week 6 to Week 7 transition

Question	Answer
Main Week 6 output	Five-page Power BI dashboard, Excel analysis workbook, and a PostgreSQL script answering the 40 assignment questions.

Question	Answer
Most important Week 6 result	A KPI set: 90.07% of billed value received, 50.38% claim approval, 11.72% admission rate, average rating 2.93.
Main issue or dependency	Only 5,000 of 60,000 bills link to an appointment; several source tables were not supplied; payment status conflicts with amounts paid.
What was tested	KPI accuracy against source, visual and table values, scope reconciliation, consistency of conclusions across segments, and visual clarity.
Relevant tracks	Data Analytics, with Data Science and Project Management as cross-track partners.
Expected improvement	Correct any mismatch, remove misleading visuals, and confirm which findings are reliable enough to pass on.

3. Part B: KPI Validation

Source values come from the independent recalculation. Dashboard values were read from the dashboard cards (screenshots dated 28 October 2026).

Page	KPI	Source value	Dashboard	Result
Executive	Total patients	4,443	4,443	PASS
Executive	Total appointments	5,000	5,000	PASS
Executive	Total revenue (billed)	771,133,439	771M	PASS
Executive	Total facilities	20	20	PASS
Executive	Total doctors	350	350	PASS
Executive	Average satisfaction	2.93	2.93	PASS
Patient	One-time patients	3,933	3,933	PASS
Patient	Emergency appointments	1,650	1,650	PASS
Patient	Returning visits	557	557	PASS
Patient	Frequent visitors (2+ visits)	510	510	PASS
Patient	Average visits per patient	1.13	1.13	PASS
Clinical	Lab tests	2,272	2,272	PASS
Clinical	Prescriptions	4,003	4,003	PASS
Clinical	Admissions	586	586	PASS
Clinical	Admission rate	11.72%	11.72%	PASS
Clinical	Average length of stay	5.66 days	5.66	PASS
Clinical	Abnormal lab results	49.74%	49.74%	PASS
Financial	Total billed	771,133,439	771M	PASS
Financial	Cash received	694,551,320	695M	PASS
Financial	Cash outstanding	76,582,119	77M	PASS
Financial	Collection rate	90.07%	90.07%	PASS
Financial	Average bill	154,227	154,227	PASS
Financial	Claim approval rate	50.38%	50.38%	PASS
Operational	Total staff	1,160	1,160	PASS
Operational	Total doctors	350	350	PASS
Operational	Total beds	2,000	2,000	PASS
Operational	Equipment items	200	200	PASS
Operational	Appointments per doctor	14.29	14.3	PASS
Operational	Appointments per bed	2.50	2.50	PASS

Result: 29 of 29 cards pass. The reconciliation query in the SQL script (Part 5) reproduced the same values, which confirms the Python results by a second route.

Notes on definitions. *(a) Claim approval of 50.38% is not the value share approved (RWF 203.2M of 407.6M is 49.9%), so it reads as a share of claims by count. The card should say so, and the definition should be confirmed against the measure. (b) The staff-by-role chart shows nurses 60.34%, doctors 30.17% and administrative 6.90%. The remaining 2.59% is 30 radiographers, by difference from 1,160. (c) Doctors appear on two pages and are counted once in the 29.*

4. Part C: Dashboard and Visual Tests

Test	Expected (from source)	Dashboard	Result
Financial matrix: 20 facilities × billed, received, outstanding (60 cells)	Source facility totals	All 60 cells match	PASS
Operational matrix: 10 departments × appointments, per doctor, admissions (30 cells)	Source department totals	All 30 cells match	PASS
Clinical matrix: diagnosis × severity admission rate (15 cells)	Source rates	All 15 cells match	PASS
Facility rows add up to card totals	771,133,439 / 694,551,320 / 76,582,119	Column sums equal the totals exactly	PASS
Billing status shares (Paid / Partially Paid / Pending)	33.81% / 33.30% / 32.89%	Same	PASS
Cash received by payment method (Card / Insurance / Cash / Mobile Money)	25.61% / 25.25% / 24.67% / 24.47%	Same	PASS
Appointments by visit type (Follow-up / Consultation / Emergency)	33.88% / 33.12% / 33.00%	Same	PASS
Staff by role (Nurse / Doctor / Administrative)	60.34% / 30.17% / 6.90%	Same	PASS
Facility slicer set to HF011	273 appointments; billed 44,206,632; rating 3.19; 17 doctors; 35 nurses	*Reading not yet recorded*	Pending
Clearing slicers restores totals	5,000 appointments	*Reading not yet recorded*	Pending

The two Pending tests have expected values calculated from source and are ready for comparison. They are listed in Appendix C.

5. Part D: Issues Found, Refinements and Retests

Resolved issues. R8 shows the value of independent testing: the check caught an error in the working data before it reached the dashboard.

ID	Issue	Expected vs actual	Cause and action	Retest
R1	Laboratory and prescription cards counted rows outside the 5,000 appointments	5,000 and 5,000 expected; raw sheets held unlinked rows	Added an inner merge with Appointments and a relationship on AppointmentID.	Cards read 2,272 and 4,003. Pass
R2	Staff counts wrong, no doctors or nurses	1,160 staff and 350 doctors expected; 80 shown	Staff table held only administrative staff. Appended the doctor, nurse, radiographer and administrative files.	1,160 and 350. Pass
R3	Appointments per doctor by department far too low	Cardiology 15.5 expected; 1.6 shown	Row field did not filter the staff table, so appointments were divided by all 350 doctors. Rebuilt on the department dimension with active relationships.	15.5 for Cardiology; all 30 cells match. Pass
R4	“New patients” card did not equal total patients	4,443 expected; 3,934 shown	Measure approximated one-visit patients. Replaced with One-time Patients and Frequent Visitors.	3,933 + 510 = 4,443. Pass
R5	Claims chart showed three segments for two claim statuses	Approved and Pending only	Legend used bill status. Changed to the claim status field.	Two segments shown. Pass
R6	Laboratory “Test” slicer on the Financial page	Financial slicers only	Slicer was copied from the Clinical page. Replaced with a FacilityID dropdown.	Confirmed. Pass

ID	Issue	Expected vs actual	Cause and action	Retest
R7	Financial page: unformatted matrix numbers, growth shown as a raw ratio, forecast too long, misspelled card labels	Consistent formats, percentage growth, short indicative forecast	Formats, measure type, forecast length and labels corrected.	Confirmed in later screenshot. Pass
R8	Validation notebook counted 60,002 appointments	5,000 expected	Billing rows had been pasted into the Appointments sheet of the working copy. Original restored and set to read-only.	Notebook returns 5,000. Pass

Open issues (found in the dashboard version of 28 October 2026). Priority is the author's judgement of how far the item can mislead a reader.

ID	Issue	Evidence	Planned action	Priority	Status
O1	Money KPIs carry no scope label, though they cover 5,000 of 60,000 bills	Part F	Add "Scope: 5,000 appointments with matched records" to the Executive and Financial pages.	High	Open
O2	Slicers show a blank "--" entry; the Patient area chart starts with "(Blank)"	Screenshots E1, E2	Add an "is not blank" filter to each; check relationships.	Low	Open
O3	Line charts are smoothed, which exaggerates month-to-month change	E1, E3, E4, E5	Turn off smoothing; start axes at zero where appropriate.	Medium	Open
O4	"Patients by Diagnosis" slices add to 4,891, not 4,443, because 448 patients have more than one diagnosis	Notebook check	Relabel as patient-diagnosis combinations, or plot appointments by diagnosis.	High	Open
O5	Referrals visual missing on the Clinical page (415 referrals in scope)	E3	Add a referral destination chart.	Medium	Open
O6	Stray Severity slicer under the Clinical matrix	E3	Remove it or move it to the slicer row.	Low	Open
O7	Facility Detail drill-through page absent	Page list	Rebuild with FacilityID as the drill-through field.	Medium	Open

6. Part E: Validated Findings

Each finding was re-tested across segments at the 5% level. "Not significant" means the gap is within normal random variation for this sample. It does not prove the groups are identical.

Ref	Finding	Test	Result	Verdict
F1	Cash collection rate 90.07%	By payment method, by facility, and on the full 60,000-row files	Methods run from 89.1% (Mobile Money) to 91.0% (Cash); facilities 88.2% to 92.2%; full files 90.1%. Method gap is about two points (Kruskal–Wallis p = 0.041).	Validated
F2	Outstanding balance	Two definitions compared	Cash basis 9.9% (RWF 76.6M). Status basis 66.2% (bills not marked Paid).	Qualified: cash basis adopted
F3	Payment status reliability	Status against bill status and amount paid	Payment status equals bill status on every bill. Amount paid is the full bill on 80.2% of bills and exactly half on 19.8%, whatever the status. Paid bills average 90.4% paid, Pending 89.7%. 323 payments marked Paid cover less than the full bill; 1,626 marked Pending have money paid.	Status field unreliable
F4	Claim approval 50.38%, no rejections	By provider	MMI 49.5%, RSSB 49.3%, Radiant 49.4%, Private 53.6%. Chi-square p = 0.214. Only Approved and Pending exist.	Validated overall; provider ranking not supported
F5	Workload per doctor, 13.3 (ICU) to 15.5 (Cardiology)	Chi-square of 5,000 appointments against an even split across 10 departments (35 doctors each)	p = 0.49. Observed 467 to 544 per department against 500 expected.	Not a reliable staffing signal
F6	Facility volume, 217 to 278 appointments	Chi-square across 20 facilities	p = 0.052, just above the 5% line.	Borderline, not conclusive

Ref	Finding	Test	Result	Verdict
F7	Seasonality	Monthly appointments, 400 to 440	p = 0.955.	No seasonal pattern; forecasts indicative only
F8	Admission rate 11.72%	By diagnosis, 10.7% to 12.4%	p = 0.66.	Validated overall; no diagnosis differs
F9	Satisfaction 2.93 out of 5	Rating distribution; by facility	Ratings 1 to 5: 20.8%, 20.1%, 22.0%, 19.7%, 17.3% (40.9% poor). Facility means 2.76 to 3.19 on 72 to 102 responses each; p = 0.795. Response rate 35.1% (1,756 of 5,000).	Validated overall; facility ranking not supported
F10	Insurance coverage	Payment method “Insurance” against claims	Insurance is the method on 1,270 bills, yet claims exist for 3,521 bills across all four methods.	Inconsistent: coverage cannot be read from payment method
F11	Facility–department mapping	Appointment department against the facility that owns it	4,744 of 5,000 appointments (94.9%) use a department belonging to another facility.	Department-by-facility views unreliable

On multiple testing. *Seven significance tests were run (F1, F4 to F9). At a 5% level, about 0.35 chance hits would be expected by luck alone. One test fell below 5% (F1, p = 0.041). It is reported as a small effect of about two points and is not used as a basis for action.*

7. Part F: Scope Reconciliation of Money Figures

Summing the whole billing and payments sheets gives much larger totals than the dashboard. The difference is scope, not error. For all 5,000 appointments, the bill amount in the billing file equals the amount on the dashboard, with 0 differences. The extra RWF 8.5 billion belongs to 55,000 bills for appointments that are not in the appointment data (APT005001 to APT060000). Loading them without their appointments would make the figures inconsistent, for example cash received of RWF 8.4 billion against billed revenue of RWF 771 million.

Measure	Whole file	In scope (5,000 appointments)	Not linked (55,000)
Bills	60,000 rows; RWF 9,309,401,961	5,000 rows; RWF 771,133,439	55,000 rows; RWF 8,538,268,522
Payments	60,000 rows; RWF 8,389,454,556	5,000 rows; RWF 694,551,320	55,000 rows; RWF 7,694,903,236
Average bill	RWF 155,157	RWF 154,227	n/a
Collection rate	90.1%	90.07%	n/a

The SQL load shows the same pattern in other tables: billing 5,000 of 60,000; payments 5,000 of 60,000; claims 3,521 of 42,036; feedback 1,756 of 21,064; laboratory tests 2,272 of 5,000; prescriptions 4,003 of 5,000; admissions 586 of 5,000; referrals 415 of 4,708.

Conclusion. The dashboard is correct for its scope. The scope must be stated on the money KPIs (O1), and the full appointment table should be requested from the data owner.

8. Part G: Updated Business Insights and Recommendations

No.	Validated insight	Evidence	Recommendation
1	About 9.9% of billed value (RWF 76.6M) is outstanding. The rate is similar across facilities and payment methods.	Parts B, E (F1)	Follow up unpaid balances systematically. Record the amount paid and the status in the same transaction so they cannot diverge.
2	Half of claims are pending: RWF 204.4M of RWF 407.6M claimed, against RWF 203.2M approved. No claim is recorded as rejected.	F4	Review pending claims with each provider and set a target turnaround. Add a Rejected status and a rejection reason to the claims data.
3	Staffing is equal across departments (35 doctors each), and workload differences are within chance variation.	F5, F6	No reallocation is justified on this evidence. Track workload monthly and revisit once the full appointment history is available.
4	Satisfaction is low overall (2.93; 40.9% poor) and does not differ significantly between facilities.	F9	Treat it as a system-wide service issue. Raise feedback coverage above 35% of appointments and record the reason for each rating.
5	The data has governance gaps: an 8.3% sample, a department–facility mismatch, unreliable payment status and missing master tables.	F3, F10, F11, Part F	Request the full appointment table and master data, correct the department–facility mapping, and define one authoritative payment field.
6	Two dashboard views can mislead: totals cover a sample, and the patients-by-diagnosis chart double counts.	O1, O4	Add scope labels, replace the chart, and hold facility league tables until more data supports the differences.

What changed since the Week 6 working view. The recommendation to move staff toward the busiest departments is withdrawn because the difference is not statistically supported. Outstanding balances are reported on a cash basis only. The facility satisfaction ranking is removed from the recommendations.

9. Part H: Cross-Track Validation (HC-POD)

The required activity runs Test, Finding, Action, Retest, Validated Improvement. Fields 2, 3 and 5 describe what Data Analytics provided. The other fields depend on the partner track's actual exchange and are left open rather than guessed.

Field	Record
1. Track collaborated with	*To complete: Data Science or Project Management*
2. Project dependency	Modelling and project decisions depend on which analytical fields and findings are reliable.
3. Component or output tested	Validated-findings and data-quality brief from Data Analytics (Parts D to F of this report).
4. Information received	*To complete*
5. Information provided	Validated findings (Part E); field-reliability warnings for payment status, department–facility mapping and the insurance indicator; the scope limitation (5,000 of 60,000 bills).
6. Testing activity completed	*To complete: how the partner checked their work against the brief*
7. Issue or finding identified	*To complete*
8. Refinement or action taken	*To complete*
9. Retest result	*To complete*
10. What changed as a result	*To complete*
11. Evidence	*To complete: screenshot or file*
12. How it improved the overall solution	*To complete*

10. Part I: Assumptions, Limitations, Risks and Dependencies

Issues R1 to R8 are resolved. The items below remain or were newly identified.

Type	Item	Impact	Mitigation	Status
Data	Only 5,000 of 60,000 bills link to an appointment	Totals describe a sample	Label the scope; request the full table	Open

Type	Item	Impact	Mitigation	Status
Data	No patient master, facility master or key linking patients or facilities to geography	No gender, age, province, patient-level insurance or facility-name analysis; province questions answered by facility	Obtain the tables	Open
Data	One year of data (2026)	No year-on-year comparison; forecasts are indicative	State this on forecasts	Accepted
Data	Highly uniform structure (35 doctors per department, 5 beds per room, 20 equipment items per department)	Looks synthetic; real differences are small	Report effect sizes honestly	Accepted
Data	All 2,000 beds are “Available”; equipment has no status	Bed utilisation and equipment availability cannot be measured	Appointments per bed used as a labelled proxy	Accepted
Data quality	Payment status conflicts with amounts paid; insurance method conflicts with claims; department–facility mismatch	Collection and coverage depend on which field is trusted	Cash basis adopted; flag to the data owner	Open
Data quality	Claim status limited to Approved and Pending	Rejection rate cannot be reported	Add a Rejected status	Open
Analytical	The Rwanda database has no attendance or no-show field	Attendance and no-show findings in the Week 7 brief cannot be reassessed on this dataset	Confirm with the mentor which dataset applies	Open
Testing	Expected values were produced by the same analyst from the same files, by two routes (Python and SQL)	Shared-misunderstanding risk reduced, not removed	Teammate cross-check (Part H)	Open
Testing	Dashboard values were read from screenshots dated 28 October	Later edits are untested	Re-test after refinements	Open

Questions a reviewer may ask

Question	Answer
The same person built and tested the dashboard.	Expected values came from the source files, not the Power BI model, and by two routes. That lowers the risk of a shared mistake but does not remove it. A teammate cross-check is set out in Part H.
Only 8.3% of bills are covered. Why trust the totals?	For all 5,000 appointments the dashboard equals the source, with 0 differences. The other 55,000 bills belong to appointments that are not in the data. Average bill differs from the full file by 0.6%, and the collection rate by about 0.05 points. The scope is to be labelled on the cards (O1).
Payment status is unreliable. Why trust 90.07%?	The collection rate compares amount paid with amount billed. It does not use status. The status basis (66.2% unpaid) was rejected for that reason (F2, F3). The full 60,000-row files give 90.1%.
You found no differences. Did the data hide them or did you miss them?	The data is uniform and looks synthetic, and this is stated. “Not significant” is reported as within chance, not as equal. Ranges are shown so the size of each gap is visible.
You ran several tests. Some will pass by chance.	Seven tests were run and one fell below 5% (F1). It is treated as a small effect and no action rests on it.
The project is about attendance. Where is it?	The Rwanda database has no attendance or no-show field. This report makes no attendance claims. Which dataset applies is to be confirmed with the mentor.
No claim was rejected. Is that good news?	It is not read that way. Only Approved and Pending exist, so the field cannot show a rejection. A Rejected status is requested.
Some items are still blank.	Yes. They are marked Pending or To complete, are not counted as passes, and are listed in Appendix C.

11. Part J: Week 8 Plan and Summary

1. Close O1 to O7, then re-run the validation notebook against the refined dashboard and record the results.
2. Add the Patient Experience page required by the brief (rating distribution, response rate, ranking by department), using F9 and its caveats.
3. Obtain the full appointment table and master data, and rebuild the model on it.
4. Share the field-reliability notes with Data Science so that payment status is not used as a feature or target without checking.

5. In the final presentation, present only validated findings (F1, F4 overall, F8 overall, F9 overall) and describe department, facility and provider differences as not significant.
6. Keep the SQL script and notebook as the reproducible validation suite for final integration.

Summary. The Week 6 dashboard is accurate for what it covers. Every card, matrix cell and chart share tested agrees with the source, and the weak points are not arithmetic. They are scope (5,000 of 60,000 bills), field quality (payment status, department–facility mapping, the insurance indicator) and a few visuals that can mislead. Eight errors were found and fixed during testing, one of them in the working data itself. The statistical checks changed the advice: differences between departments, facilities and providers are mostly within chance, so the staffing reallocation is withdrawn. What the data does support is a collections follow-up, a review of the pending claims backlog, better feedback coverage and cleaner data governance.

Readiness for Week 8: ready for final integration, subject to closing O1 to O7, the two Pending slicer tests and the cross-track record.

Appendix A: Week 6 Deliverables Carried Into This Submission

Deliverable	Status	Reference
SQL script answering the 40 questions	Included. Run on PostgreSQL 16 without errors; results checked against independent calculations; the reconciliation query reproduces the dashboard KPIs.	HealthConnect_Week6_SQL_Script.sql, with prepare_csv_for_postgres.py
Database design, cleaning and validation	Included in the script: schema with keys and constraints, staging tables, load reconciliation and 12 data-quality checks.	Script Parts 1 to 3
Power BI dashboard (.pbix), five pages	Included and tested in this report.	*To complete: attach the .pbix*
Executive dashboard PDF	*To complete: export from Power BI*	*To complete*
Business insights report	Parts E to G of this report.	This report
PostgreSQL database backup	*To complete: after loading, run pg_dump -U postgres -d healthconnect -F c -f healthconnect.backup*	*To complete*
README and GitHub link	*To complete*	*To complete*

Notes on the SQL script. *Questions 2 to 4 and 7 to 10 need a patient master table, which was not supplied. They are written against an optional hc.patients table and return no rows until it is loaded (tested on sample data). Question 1 (registered patients), Question 5 (chronic conditions) and Question 6 (insured patients) are answered from appointments, diagnoses and claims. Question 18 is answered by facility because no province link exists. Question 33 is answered by service line (visit type) because bills are not linked to individual services.*

Appendix B: Evidence Index

Ref	Evidence	File
E1	Executive dashboard (28 October)	Screenshot_2026-10-28_120635.png
E2	Patient Analytics page	Screenshot_2026-10-28_120708.png
E3	Clinical Analytics page	Screenshot_2026-10-28_120754.png
E4	Financial Analytics page	Screenshot_2026-10-28_120824.png
E5	Operational Analytics page	Screenshot_2026-10-28_120859.png
E6	Python validation notebook, output from a clean run	HealthConnect_W7_Validation.ipynb
E7	Testing record workbook	HealthConnect_W7_Testing_Record.xlsx
E8	SQL script and run output	HealthConnect_Week6_SQL_Script.sql
E9	Before and after screenshots for R1 to R7	*To complete: insert from the build sessions*
E10	Cross-track evidence	*To complete*

Appendix C: Items Still Open Before Submission

These are the only blanks in the report. None is counted as a pass.

1. Set the facility slicer to HF011 and record the five values shown (Part C).
2. Clear the slicers and record the appointment total (Part C).
3. Complete cross-track fields 1, 4 and 6 to 12 from the actual exchange, and attach the evidence (Part H, E10).
4. Attach the .pbix, the Executive dashboard PDF, the database backup and the README or GitHub link (Appendix A).
5. Insert before and after screenshots for R1 to R7 (E9).
6. Confirm with the mentor which dataset applies to attendance analysis (Part I).